



PREREFERRAL STABILIZATION



1 STABILIZE PRIOR TO TRANSPORT



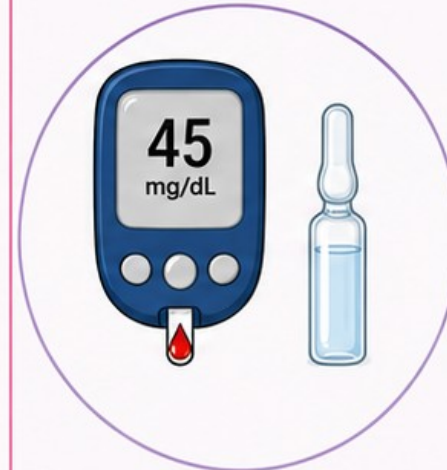
2 COMMUNICATE TO REFERRAL CENTRE



3 MAINTAIN WARM CHAIN



4 PREVENTION AND MANAGEMENT OF HYPOGLYCAEMIA



5 PREVENTION AND MANAGEMENT OF HYPOXIA



**Stabilize
prior to
transport**

T

Warm the baby (Skin-to-skin contact, Keep baby with mother, wrap the baby)

A

Suction the airway if essential

B

Oxygenate if needed

C

Give/arrange medications as per physician's order

Communication

1

Identify facility
of referral

2

Inform the
nursing officer
& MO in-
charge (over
phone)

3

Document
inter-facility
referral slip

4

Sign the
referral slip &
send with
mother/caregiv
er

**Temperature Maintenance
as per warm
chain**

Prevention & Management of hypoglycaemia



Procedure for checking glucose-heel prick, glucose cut-off levels- <45 mg/dL



If baby is able to feed,
continue breastfeeding, OR



Give Expressed Breastmilk



**Or 2 ml/ kg 10% Dextrose
Solution through Orogastric
tube**



HEEL PRICK OF NEWBORN BABY

FOR RBS (BLOOD GLUCOSE) MONITORING



- 1** Identify baby and explain procedure.



- 2** Perform hand hygiene and wear gloves.



- 3** Warm the heel.



- 4** Select lateral heel site.



Puncture the lateral aspect of the heel, avoiding the middle portion.

- 5** Clean and dry skin.



- 6** Prick heel with sterile lancet.



- 7** Wipe first drop.



- 8** Collect blood on test strip.



- 9** Record glucose reading.



- 10** Apply pressure and comfort baby.



- 11** Dispose sharps safely.



- 12** Document result and action taken.



Prevention & Management of Hypoxia

1

Suction
mouth & nose

2

Position
ensuring clear
airway

3

Use pulse
oximeter

4

Give Oxygen
by nasal
prongs

5

Maintain
oxygen
saturation at
>90%

Danger signs of newborn



Poor feeding



Apnoeic attacks
or cyanosis

Respiratory difficulty,
apnoeic attacks or cyanosis



Undue lethargy



Sudden rise or fall in
body temperature



Appearance of jaundice within 24 hours of
age or yellow staining of palms or soles
or persistent jaundice beyond 2nd week of life



24h
meconium ✗

48h
urine ✗

Failure to pass meconium within
24hours/ urine within 48 hours



persistent vomiting



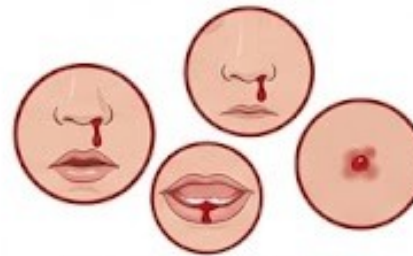
Diarrhoea



drooling of saliva or choking
during feeding



Excessive crying/seizures



Bleeding from any site



Evidence of infections (Pustules, Stiff
Neck, Readiness/ pus from umbilical
stump, Bulging of fontanell etc)